

Standard Operating Procedure

Clinical Audit Cycle

«Hospital Name» — Supports PSQ.5.a-f

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By Quality Coordinator Name: _____ Signature: _____			
Approved By Medical Superintendent Name: _____ Signature: _____			

1. Purpose

This SOP describes how «Hospital Name» plans, runs, documents and closes the loop on a clinical audit, so every clinical department is audited at least once every two years.

2. Scope

Applies to every clinical audit undertaken by any clinical department.

3. Responsibility

Quality Coordinator: schedules audits and ensures the cycle is closed.

Audit Team (clinicians, administrators, nurses): conducts the audit against predefined parameters.

4. Procedure

1. Schedule one clinical audit per clinical department at least once every two years, matching the department's scope of services. A topic can be disease-based, cost-based, community-based, or based on morbidity such as length of stay.

2. Before starting, confirm this is genuinely an audit and not research. An audit measures current practice against an existing, already-accepted standard, and does not change the care patients receive — it only checks compliance and prompts improvement. Research tests a new question or hypothesis, may involve an

intervention beyond routine care, and is intended to generate new, generalisable knowledge. If patients would receive anything other than the hospital's standard care, or the goal is new knowledge rather than checking compliance, this is research and needs ethics committee review instead of this SOP.

3. Set the objectives and the standard the audit will measure against, build a checklist where useful, and define the sampling and data-collection method before starting — this avoids bias creeping in after the results are seen.

4. Identify participants — a mix of clinicians, administrators and nurses, for example members of the core or quality assurance committee.

5. Maintain patient and staff anonymity throughout — names must not appear in the report or be mentioned in any discussion or conference about the findings.

6. Record findings on the checklist, then prepare a report highlighting the key findings.

7. Implement remedial measures for any gap found — preferably based on root-cause analysis (see the Incident Reporting & RCA SOP for how to run one).

8. Re-measure after the remedial action to confirm the gap has actually closed. The audit cycle runs: set the standard, measure current practice, compare, implement remedial action, then re-measure — it is not complete until that final re-measurement is done and documented.

5. Records

Supported by Records 1-6 of the PSQ.5 Required Records pack — the audit schedule, parameter definitions, participation record, anonymity compliance record, audit report, and the remedial action/re-audit record.

6. Related Documents

Policy on Clinical Audit (HCO/PSQ/POL/05) — PSQ.5 policy

7. Review

Reviewed annually, or immediately if an audit reveals a gap in this procedure.

8. References

National Institute for Health and Care Excellence, Healthcare Quality Improvement Partnership, Royal College of Nursing. Principles for Best Practice in Clinical Audit. Radcliffe Medical Press.